INLAND EMPIRE MEDICAL NETWORK, INC.

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MOLLUSCUM CONTAGIOSUM TREATMENT & CONSENT

What is Molluscum Contagiosum? Molluscum contagiosum is a common skin disease caused by a virus which affects the top layers of the skin. The name molluscum contagiosum implies that the virus develops growths that are "contagious", ie., easily spread by skin contact. Similar to warts, this virus belongs to the the poxvirus family and enters the skin through small breaks in the skin or hair follicles. It does not affect any internal organs. Only 20% of molluscum eruption may resolve spontaneously without treatment after 1-2 years as the immunity will develop.

How do you get molluscum? The molluscum virus is a DNA virus that is transmitted from a direct contact with the skin of one person who has these growths to the skin of another person that has small breaks in the skin. This occurs most often in cases where skin-to-skin contact is frequent, in young children, among siblings, or in swimming pools or daycare centers. If growths are present in the genital area, molluscum can be sexually transmitted or self-innoculated by contact with the infected skin areas of hands or fingers.

Who is most susceptible to get molluscum? Young children are more susceptible because their immune system has not yet encounter the molluscum virus, therefore not yet develop an immunity against the virus. Children with a history of atopic eczema or compromised skin barrier functions have an increased risk of developing these growths. Molluscum infection is more commonly seen in young children, and rarely in adults (only in those who have suppressed immune system). Also tropical climates where there are ample warmth and humidity seem to favor the growth of this virus.

How is molluscum treated? Most studies show that molluscum contagiosum infection can resolve spontaneously after months of exposure. However, most clinicians and parents do not observe this spontaneous resolution, until some form of treatment is initiated.

Treatment of Molluscum :

- Treatment of Molluscum may include **freezing with LN_2 , curettage**, or **application of cantharidin** (Canthacur).
- Freezing with LN_2 method can be painful and scary for most children, but can be effective.
- Cantharadin is a strong vesicant (beetle blistering agent) that will produce a **blister** on and around the lesion in order to lift the viral component of the lesion away from the normal skin and initiate an inflammatory immune response to the viral lesion. **Patients vary in their sensitivity to cantharadin, including itching, tingling, burning, minor or extremely tenderness may develop from the inflammatory response**.
- After the clinician applies cantharadin on the lesions, a white film will form over the lesion. This white film must be washed off within 5-8 hours depending on the thickness of the skin area. **You will be instructed to wash areas off in ..... hours with warm water.**
- If treated areas become too painful, Children's Motrin may be given.
- If small blisters develop, allow them to dry up on their own after few days; or you may poke the blister roof with sterilized needle to release the underlying fluid, then leave the blister roof intact. DO NOT REMOVE THE BLISTER ROOF. You may apply triple antibiotic ointment (eg., Polysporin, Neosporin) or Aquaphor Ointment to open lesions. If large blisters develop, call our office for instructions or schedule urgent follow-up appointment (909-581-6732).
- If itching occurs, moisturize patient's skin with Aquaphor ointment, Cerave, Cetaphil, or Aveeno lotion, plus any prescription (steroid) creams given by the doctor, if needed.

I, the undersigned, have read and fully understand and consent to the aforementioned treatment of molluscum. I understand the risks and possible side effects of treatment, and the instructions how to handle the potential side effects.

AUTHORIZED PERSON (Relationship to patient) DATE WITNESS / PROVIDER DATE